INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Gemcitabine alone · non-small-cell lung cancer

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
28 days
See the active treatment phase

Treatment stages
2
Explained in the treatment reference

Regimen source
NCCP 00284
Version 5

Every 28 days

Treatment stages

01 **Repeated 28 day cycles**
3treatment visits per cycle

02 **Review benefit and recovery**
Next plan agreed

Your team confirms the active stage and dates. The full treatment guide explains each step.

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call oncology immediately for fever or sudden illness, breathing difficulty, chest pain, uncontrolled vomiting, bleeding, very little urine, severe headache or confusion, or a spreading/ blistering rash. Tell the nurse about infusion-site pain.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

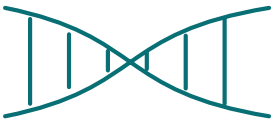
Your medicines

Gemcitabine aims to control lung cancer and its symptoms. Treatment is reviewed against side effects, your health and your priorities; benefit is not guaranteed.

CHEMOTHERAPY

Gemcitabine

Antimetabolite chemotherapy



Interferes with deoxyribonucleic acid (the cell's genetic material) (DNA) building

Gemcitabine interferes with making DNA, which dividing cancer cells need. Healthy blood-forming cells and other tissues can also be affected.

DNA building → Replication disrupted → Dividing cells affected

What to watch for

Report fever, unusual bleeding, breathlessness, rash, swelling or reduced urine promptly.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Understanding the evidence

Gemcitabine has been studied in older adults with advanced lung cancer.

Small historical single-arm older-adult study cannot establish a comparative survival benefit or predict an individual outcome.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Your overall treatment

Treatment stages

01 **Repeated 28 day cycles**
3treatment visits per cycle

02 **Review benefit and recovery**
Next plan agreed

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

3TREATMENT VISITS PER CYCLE

Repeated 28 day cycles

1 2 **Further cycles as agreed**

Gemcitabine is given on Days 1, 8 and 15. Week 4 has no planned dose. Each cycle lasts 28 days. For lung cancer there is no fixed six-cycle limit in this source: continue only while the team considers treatment helpful and tolerable.

NEXT PLAN AGREED

Review benefit and recovery

Review

Your team decides when to scan and whether to continue, pause or change treatment. Surgery and radiotherapy are separate plans.

Lung treatment duration is individual; the pancreatic six-cycle cap is not transferred here.

BASELINE → TREATMENT → FOLLOW-UP

My tests and checks

Required safety checks are distinct from individual response imaging and follow-up. No routine tumour-marker interval is specified.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00284 v5 · official regimen

THE WHOLE TREATMENT JOURNEY

Gemcitabine alone · non-small-cell lung cancer: treatment and checks

Locally advanced or metastatic non-small-cell lung cancer (NSCLC) in older adults or people with ECOG2

Gemcitabine is given on Days 1, 8 and 15. Week 4 has no planned dose. Each cycle lasts 28 days. For lung cancer there is no fixed six-cycle limit in this source: continue only while the team considers treatment helpful and tolerable.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before treatment	Treatment days	Between doses	Next cycle	After treatment
Treatment NCCP 00284	Confirm lung indication	Gemcitabine Days 1/8/15	No dose Week 4	Repeat only if appropriate	Agree next plan
Blood tests NCCP 00284	full blood count (FBC), kidneys, liver	FBC before every dose	Extra tests if unwell	Renal/liver before cycle	Follow abnormalities
Other checks NCCP 00284	Audiometry/ creatinine clearance (CrCl) if indicated	Review symptoms	Call when unwell	Review benefit and toxicity	Individual scans/follow-up

What each check is for

1 Prepare

Before first dose

Gemcitabine aims to control lung cancer and its symptoms. Treatment is reviewed against side effects, your health and your priorities; benefit is not guaranteed.

Ask / act: Confirm aim, prescription, tests and contacts.

2

Review

Before each dose

Check treatment remains appropriate

Ask / act: Report symptoms and confirm blood results before dosing.

3 Next plan

At agreed review points

Assess benefit and recovery

Ask / act: Confirm scan and follow-up dates with the team.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Before each visit

Bring your medicine list and report illness. The team reviews blood results before deciding the dose.

STEP 2

Three treatment visits

Day 1,Day 8 and Day 15 are planned infusion visits. Week 4 has no routine dose.

STEP 3

At home

Keep the prescribed support medicines and urgent contact plan available. Do not make up a missed dose yourself.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 28: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Gemcitabine only	 **Day 1** Day 1 only; after safety checks.
Gemcitabine only	 **Day 8** Day 8 only; after safety checks.
Gemcitabine only	 **Day 15** Day 15 only; after safety checks.
Symptom awareness	 **Days 1–28** Symptoms may occur on any day; this is not a personal risk curve.

Timing varies: Tiredness, bowel changes and infection can occur between visits. Blood tests and your symptoms guide dose decisions; there is no guaranteed safe day.

Changes to the plan: The team may delay, omit or reduce doses for safety. Do not add a missed dose yourself.

> **Why these bands stay broad:** Relative treatment days, without personal dates. Symptoms are not confined to a fixed day. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00284 v5 · official regimen ↗

> **There are no guaranteed "safe days".** Call oncology immediately for fever or sudden illness, breathing difficulty, chest pain, uncontrolled vomiting, bleeding, very little urine, severe headache or confusion, or a spreading/blistering rash. Tell the nurse about infusion-site pain.

Between-treatment notes

Day 1: Gemcitabine only

Treatment after blood-test and symptom review. Your team confirms whether this dose can proceed.

Day 2–7: At home

No planned chemotherapy on these days. Use only the support medicines prescribed for today. Call using your contact plan if unwell; do not wait for a treatment day.

Day 8: Gemcitabine only

Day 9–14: At home

Day 15: Gemcitabine only

Day 16–28: At home

Support around the medicines

1 **Sickness medicines**

Gemcitabine has low sickness risk. Follow the prescribed preventive or rescue medicine plan; do not take a platinum anti-sickness course for gemcitabine alone.

2 **Feeling unwell**

Do not dismiss fever as a flu-like drug effect. Call the oncology team immediately for fever, shaking chills or sudden illness.

3 **Medicines and fluids**

Follow your individual drinking advice. Report reduced urine, swelling, breathlessness or trouble drinking, and ask before new medicines or supplements.

Questions worth asking

Why are there three visits?

The same medicine is planned on Days 1, 8 and 15, followed by a week without a planned dose. Each visit has its own safety checks.

Is treatment always six cycles?

No. That fixed duration belongs to the separate pancreatic adjuvant indication. Your lung treatment continues only while appropriate.

When will I have scans?

Your consultant agrees timing from your disease and treatment goals; this regimen does not mandate one interval.

Try explaining the plan back: Explain back your treatment days, checks and whom to contact when unwell.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Gemcitabine alone · Days 1, 8, 15

Active medicines: gemcitabine

Emetogenic risk: low

Medicine	Dose / route	When
Dexamethasone	8 mg orally	Before each gemcitabine dose if prescribed NCIS low-risk reference; individualise and reconcile with any other steroid course.

Additional phase-specific support

- Gemcitabine has low sickness risk. Follow the prescribed preventive or rescue medicine plan; do not take a platinum anti-sickness course for gemcitabine alone.
- Do not dismiss fever as a flu-like drug effect. Call the oncology team immediately for fever, shaking chills or sudden illness.
- Follow your individual drinking advice. Report reduced urine, swelling, breathlessness or trouble drinking, and ask before new medicines or supplements.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Head / alertness

Gemcitabine

Urgent advice for severe headache, confusion, seizures or vision change.

3 Mouth / sickness

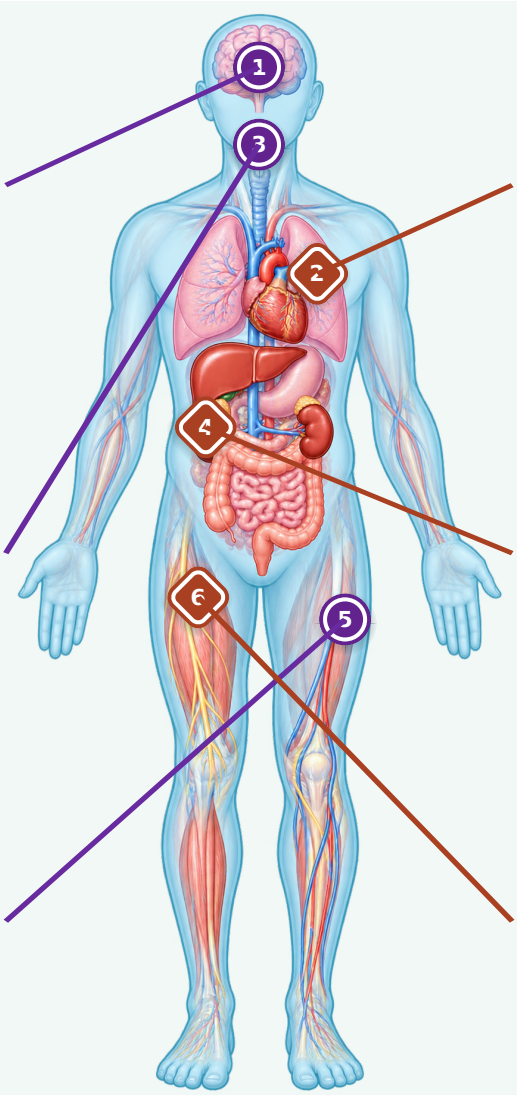
Gemcitabine

Report mouth sores, vomiting or trouble drinking.

5 Skin

Gemcitabine

A spreading rash, blisters or peeling skin needs urgent review.



2 Breathing

Gemcitabine

New or worsening breathlessness needs urgent advice.

4 Kidneys / fluids

Gemcitabine

Call for much less urine, swelling or sudden illness.

6 Blood cells

Gemcitabine

Fever, infection symptoms or unusual bleeding need prompt advice.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Call oncology immediately for fever or sudden illness, breathing difficulty, chest pain, uncontrolled vomiting, bleeding, very little urine, severe headache or confusion, or a spreading/blistering rash. Tell the nurse about infusion-site pain.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Your record and contact card

IN THIS CHAPTER

Space for your own cycle or review record, questions and notes, followed by your treatment card.

Record each cycle separately. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

Use a new record sheet for each cycle or review. Copy or print another sheet when needed; the number of sheets does not prescribe the length of treatment.

Cycle number: _____ Start date: _____ Next review: _____

Use another record for each new cycle. Record actual dose dates and omitted doses; the record is not a fixed course-length target.

Current 28 day cycle · Days 1, 8, 15 · copy this sheet for another cycle of the same phase.

Gemcitabine · Chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Fatigue	□ Usual energy	□ Tiredness improves with rest	□ Rest does not help; usual tasks affected: call	□ Self-care difficult: urgent review	□ Collapse: 112/999
Nausea	□ None	□ Queasy but eating and drinking	□ Intake reduced: contact the team	□ Unable to maintain intake: urgent review	□ Collapse / severe dehydration: 112/999
Mouth soreness / ulcers	□ None	□ Mild soreness; eating normally	□ Painful; soft food needed: call	□ Cannot eat / drink enough: urgent review	□ Major breathing difficulty: 112/999
Bleeding / bruising	□ None	□ New small bruise / brief bleed: report	□ Repeated bleeding: call today	□ Bleeding that will not stop: urgent help	□ Heavy bleeding / collapse: 112/999

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Keep your treatment details to hand

Locally advanced or metastatic NSCLC in older adults or people with ECOG2 · NCCP 00284 v5 · Document revised 4 November 2026

Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

FRONT / CONTACTS

My Treatment Card

SACTCheck

Name: _____

Gemcitabine

Days 1, 8, 15; repeat every 28 days. Duration agreed with your team.

Phase: _____ Cycle plan: team to confirm

Daytime (EXAMPLE) 087 12345678
Out of hours (EXAMPLE) 087 87654321

Unwell or fever? Call oncology now.
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

BACK / TREATMENT RECORD

My cycle record

Rolling record — follow your agreed treatment plan.

Cycle	Date / initials

My treatment information

Latest treatment date: _____

Plan changed? Ask the team to update this card.

CUT outer border

FOLD centre — do not cut

Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

Your local contacts and resources

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Hospital / team: _____
Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

SACTCheck v0.84.32 · NCCP 00284 v5 · Wallet-card sheet
Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

CHAPTER 05

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00284 v5 · official regimen**
   https://healthservice.hse.ie/documents/6699/284_v5_Gemcitabine_Monotherapy_28_day.pdf
2. **Gemcitabine product information**
   https://www.medicines.org.uk/emc/product/3119/smpc
3. **Ricci et al. 2000: gemcitabine in older adults with advanced non-small-cell lung cancer (NSCLC)**
   https://pubmed.ncbi.nlm.nih.gov/10688490/
4. **NCCP Antiemetic Medicines for inclusion in NCIS (Medical Oncology) · v8, 10 February 2025**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf